

THE CHRONICLES OF **RAMIPRIL**

Survival Analysis & Risk Reduction: A Tale of the HOPE Study

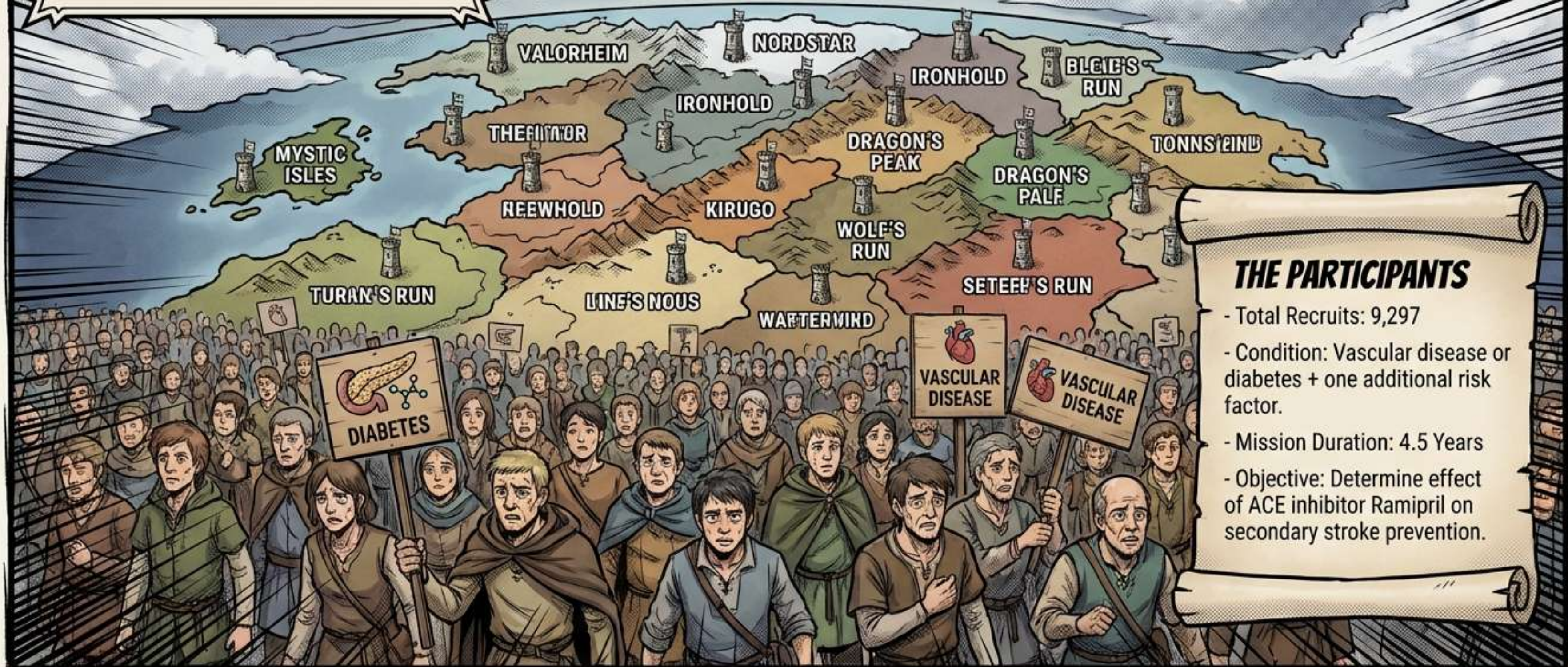
**THE
STROKE**

Based on Bosch, J., Yusuf, S., et al. (2002).
Use of ramipril in preventing stroke:
double blind randomised trial.

In the Kingdom of
Vascular Health, a
dark menace rises.
Can the Gold Knight
prove his worth?

THE GATHERING STORM

The HOPE Alliance summons 9,297 souls to the arena. These are not ordinary citizens; they are marked by the curse of High Risk.

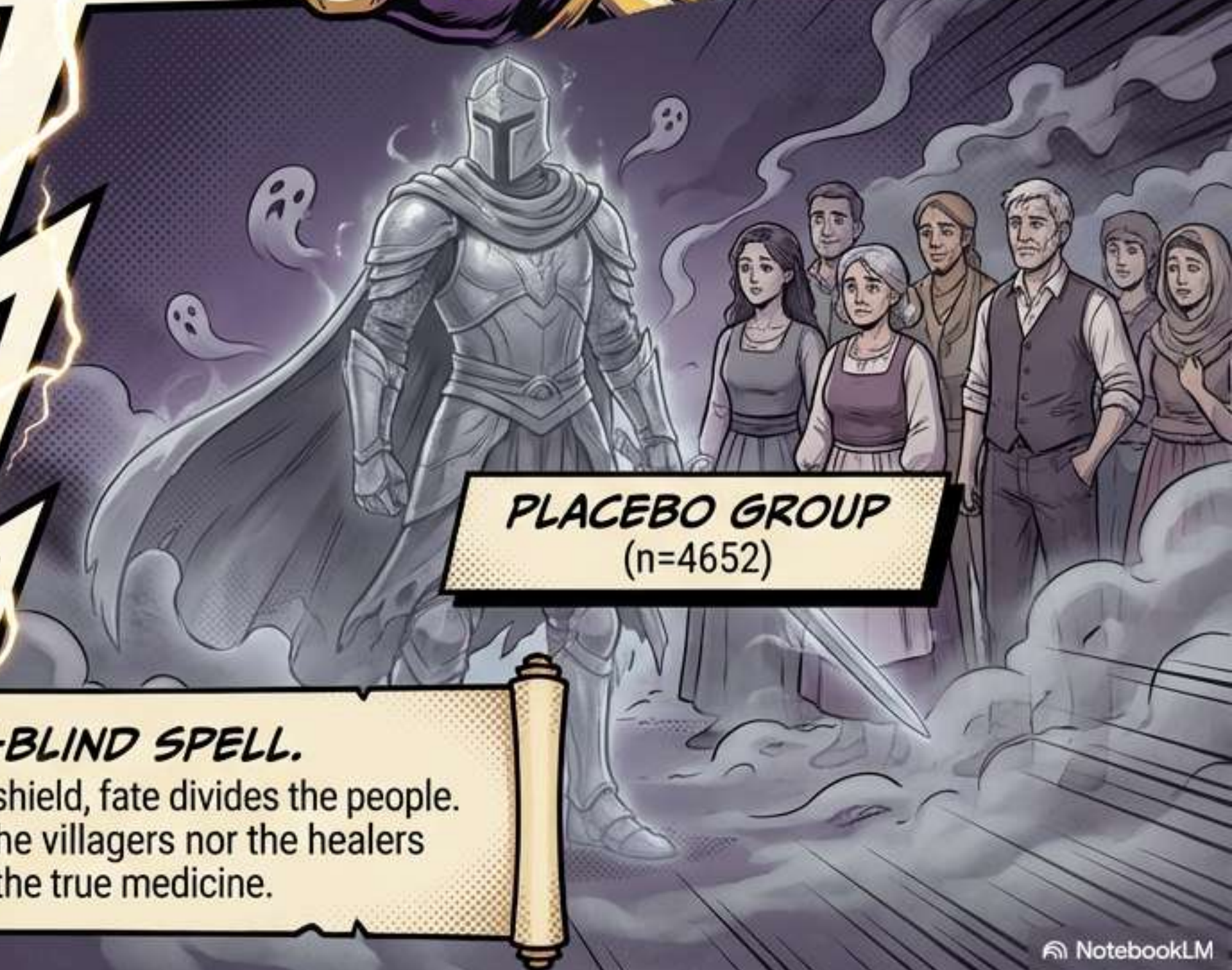


THE PARTICIPANTS

- Total Recruits: 9,297
- Condition: Vascular disease or diabetes + one additional risk factor.
- Mission Duration: 4.5 Years
- Objective: Determine effect of ACE inhibitor Ramipril on secondary stroke prevention.



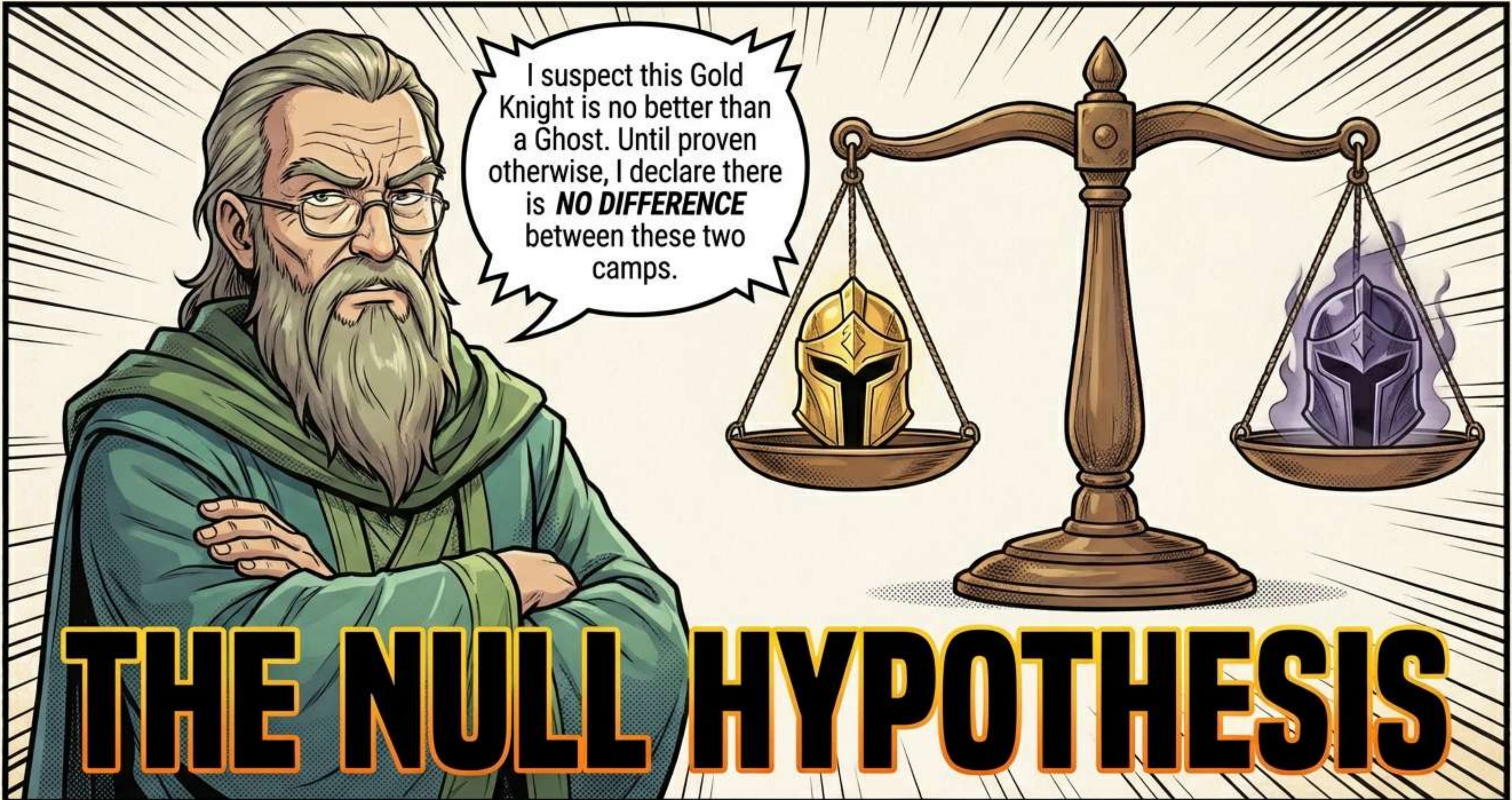
RAMIPRIL GROUP
(n=4645)



PLACEBO GROUP
(n=4652)

THE DOUBLE-BLIND SPELL.

To test the true power of the shield, fate divides the people.
A fog descends—neither the villagers nor the healers
know who holds the true medicine.

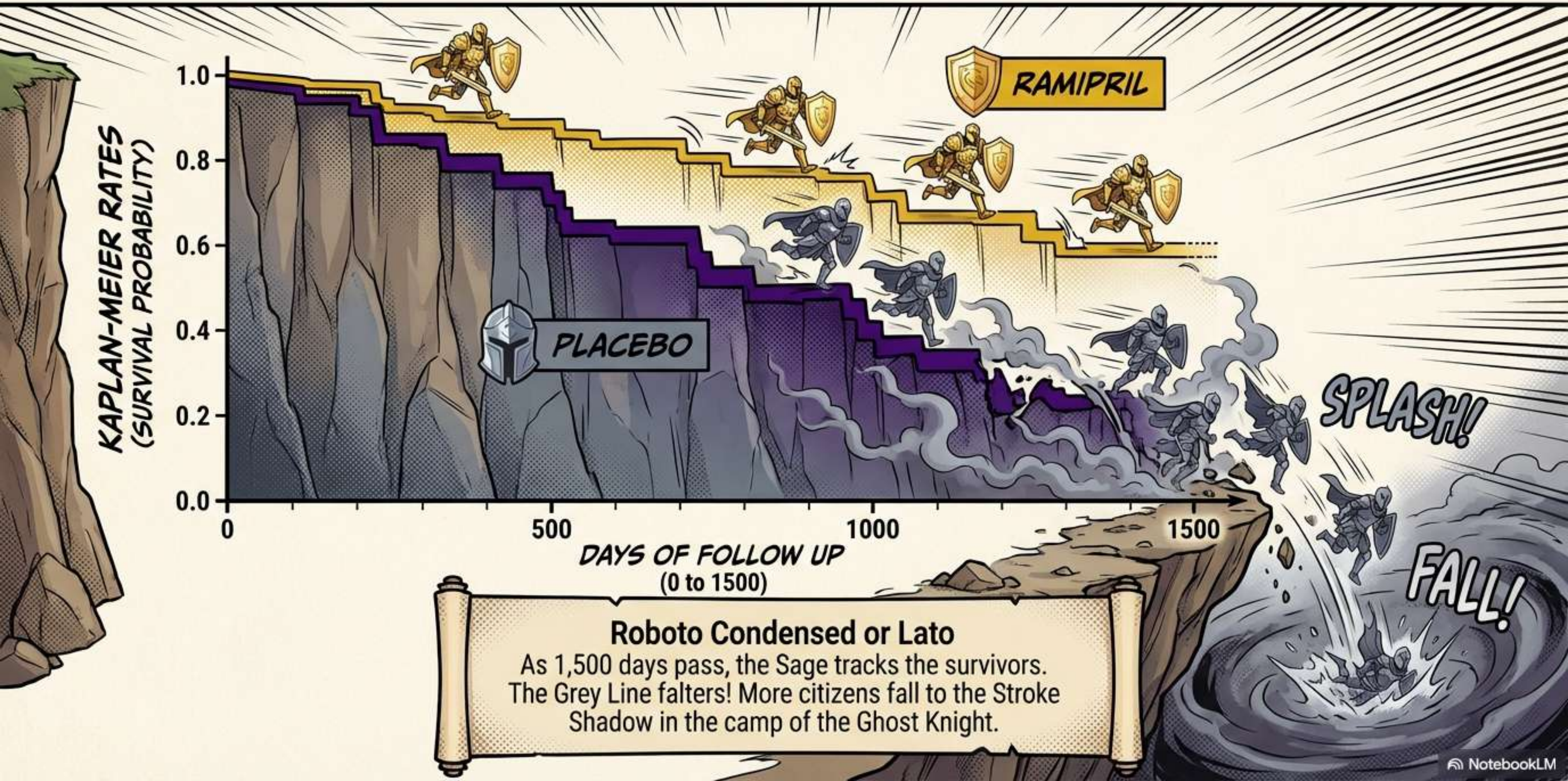


I suspect this Gold Knight is no better than a Ghost. Until proven otherwise, I declare there is **NO DIFFERENCE** between these two camps.

THE NULL HYPOTHESIS

The burden of proof lies with the hero. If the difference in survival is small, the Null Hypothesis stands undefeated.

THE BATTLE OF THE CLIFF (KAPLAN-MEIER PLOT)



Is this gap real? Or just a trick of the light? The 'Log Rank Test' is summoned to judge the statistical distance between the shields.

LOG RANK TEST!

Survival distributions between groups are compared using the log rank test to determine significance.

CASUALTY REPORT (STROKE EVENTS)

RAMIPRIL CAMP

Total: **4645**
Strokes: **156**
Rate: **3.36%**

PLACEBO CAMP

Total: **4652**
Strokes: **226**
Rate: **4.86%**



The dust settles after 4.5 years.
The Gold Knight has protected more citizens.
The raw numbers show a clear victor.



Impossible!
The probability of
this happening
by luck is only 2
in 10,000!
The Null Hypothesis
is destroyed!

Significance Level: A P-value of 0.0002 means the result is highly significant. It is extremely unlikely to be due to chance.

COX REGRESSION ANALYSIS

**PLACEBO
RISK (1.0)**



**RAMIPRIL
RISK (0.69)**



RISK RATIO = 0.69
95% CONFIDENCE INTERVAL: 0.56 – 0.84

The Cox Regression technique analyzes the “Hazard.” It reveals that the risk in the Gold Camp is only 0.69 times the risk in the Ghost Camp. Since the Confidence Interval does not cross 1.0, the victory is real.



Calculating RRR (Relative Risk Reduction):
Formula: $(\text{Placebo Rate} - \text{Ramipril Rate}) / \text{Placebo Rate}$
Math: $(4.86 - 3.36) / 4.86 = 0.31$
Result: 31% RRR

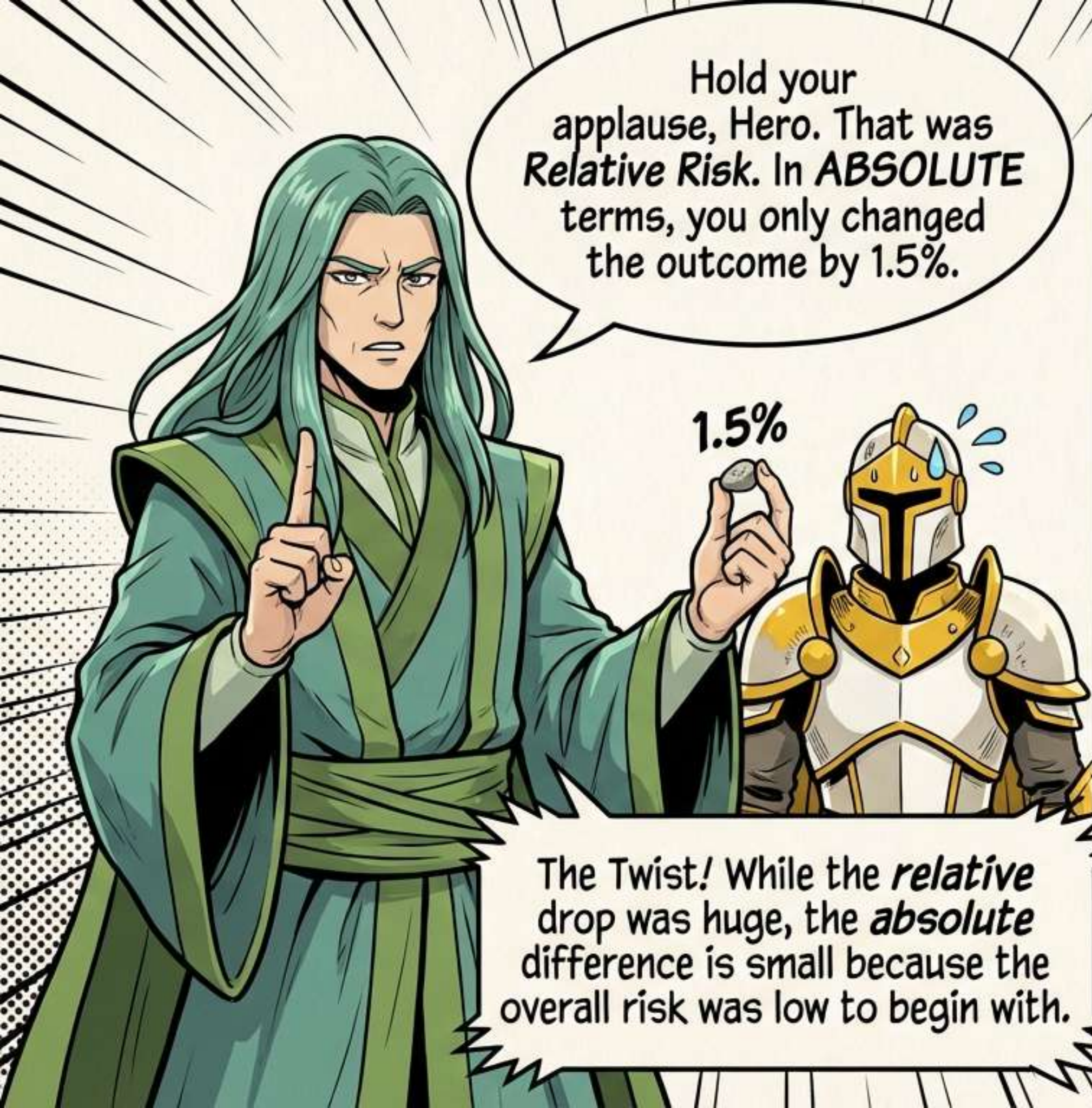
FATAL STROKE STATS



Ramipril Deaths: 17 (0.37%)
Placebo Deaths: 44 (0.95%)
Risk Ratio: 0.39

FATAL STROKE RRR = 61%

Against the deadliest blow, the shield is strongest.
The relative risk of fatal stroke was reduced by 61%.



Hold your
applause, Hero. That was
Relative Risk. In **ABSOLUTE**
terms, you only changed
the outcome by 1.5%.

1.5%

The Twist! While the *relative*
drop was huge, the *absolute*
difference is small because the
overall risk was low to begin with.

Absolute Risk Reduction (ARR)

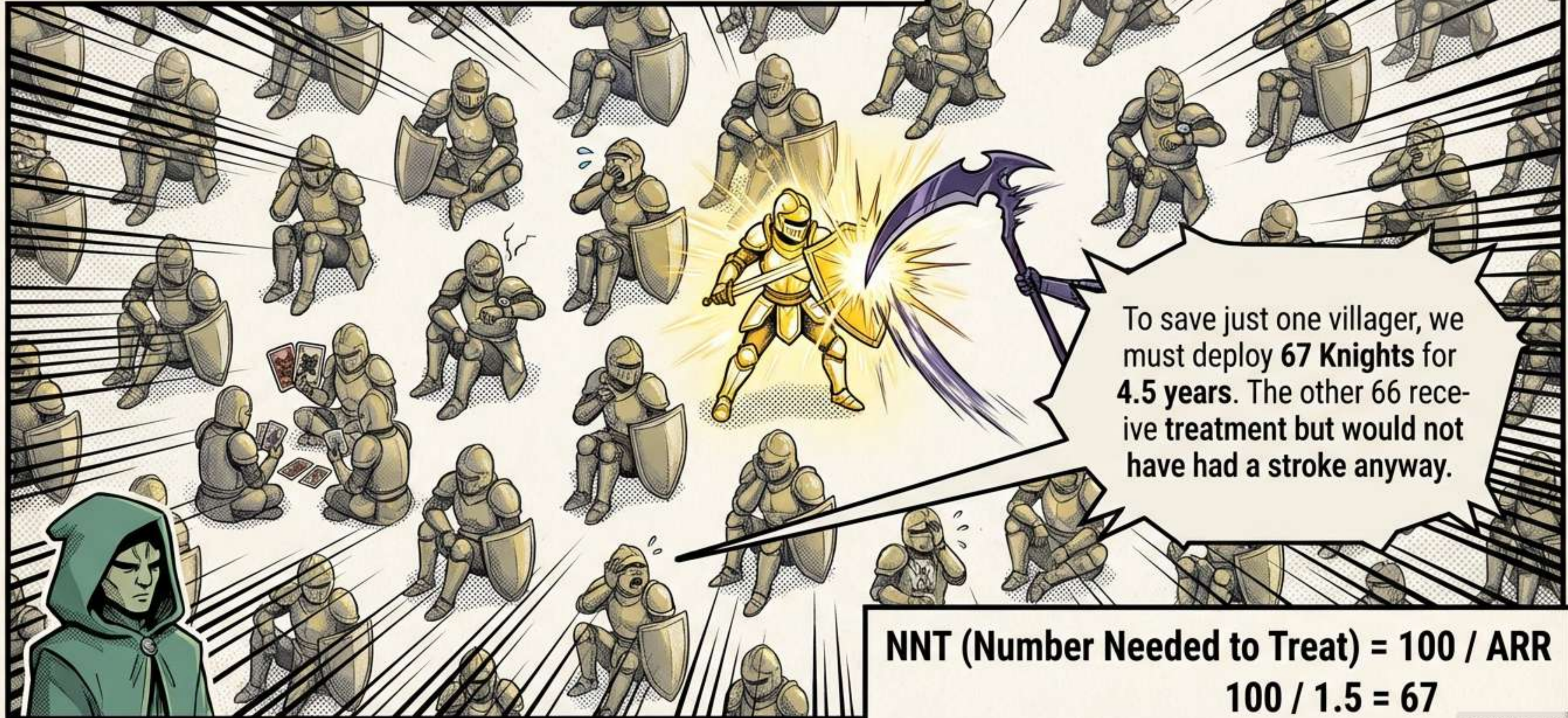
Placebo Rate: 4.86%

Ramipril Rate: 3.36%

Calculation:

$$4.86\% - 3.36\% = \underline{\underline{1.5\%}}$$

THE COST OF WAR: NNT



THE CLINICIAN'S DILEMMA

31% Stroke Reduction! And it works even though Blood Pressure only dropped 3.8 mmHg!

Treat 67 people for years to save one? Is it worth the cost and side effects?

This is the core conflict of Evidence-Based Medicine. **RRR** and **NNT** from the same study can drive opposing prescribing habits.

THE LEGEND RECAP:



1. **Hypothesis:** Ramipril prevents stroke in high-risk patients.



2. **Analysis:** Kaplan-Meier curves & Log Rank Test showed significant survival difference ($P < 0.0002$).

3. **Victory:** Relative Risk Reduction (RRR) = 31%.%



4. **Reality:** Absolute Risk Reduction (ARR) = 1.5%.

5. **Cost:** NNT = 67.



Full Citation: Bosch, J., Yusuf, S., Pogue, J., Sleight, P., Lonn, E. et al. (2002). Use of ramipril in preventing stroke: double blind randomised trial. **BMJ** 324: 699–702.